

Gestational Diabetes Mellitus

-Aashna

Name: Mrs. ABC

Age: 25 years

Occupation: Teacher

Educational status: Post Graduate

Husband's name: XYZ ; Age: 26 yrs. Occupation: Teacher

Socio-economic status: Upper Middle Class

LMP: 30/7/2019 EDD: 7/5/2020

Gravida: primigravida

Para 0 Gestational age: 26 weeks 6 days

Date of admission: 4/2/2020 Date of examination: 6/2/2020

- Chief Complaints: The lady came for regular ante-natal checkup and was found to have high blood sugar levels on evaluation

History Of Present Pregnancy • 1st Trimester Pregnancy was detected by urine pregnancy test following 35 days of missed period History of spontaneous conception 1st trimester scan was done and found to be normal Weight gain in 1st trimester was 800 gms Folic acid tablets were taken

No h/o burning micturition

No h/o fever with rashes, excessive vomiting

No h/o spotting or bleeding per vagina

No h/o pain abdomen

No h/o exposure to radiation and drug intake

Regular ANC was done

2nd trimester: She felt quickening at 5th month and continued to perceive fetal movements well

She was found to have increased blood sugar during her next ANC visit and was advised to have proper regular diet and was put on insulin.

She discontinued the insulin medication against doctors advice. During her next ANC her blood sugar values were still high.

2nd trimester scan was done and found to be normal.

Weight gain in 2nd trimester is 3kg

2 doses dT were taken.

Iron and calcium tablets taken.

No h/o increased frequency of micturition and increased thirst

No h/o pedal edema, blurring of vision, headache.

No h/o increased appetite

No h/o burning micturition

No h/o of pain abdomen, leaking or bleeding per vagina

Past obstetric history

Primigravida

Menstrual history LMP: 30/4/2020

EDD: 7/3/2021

Age of attainment of menarche – 11 years

Regular cycle of 28 to 30 days, flow for 4-5 days

Changes 4 pads per day

Not associated with dysmenorrhea, No h/o of passage of clots

- Past Medical History: Patient has no previous history of diabetes or hypertension earlier in life.
- Marital history: Married at the age of 19, married life of 5 years. Non consanguineous marriage

- Family History: Both mother and father are diagnosed with type2 diabetes mellitus.

- Personal history

Diet – mixed

Appetite – normal

Sleep - adequate

Bowel and bladder – regular

No history of substance abuse

No history of any known drug allergy

No history of use of any contraceptives

On Examination:

Patient is conscious, cooperative well oriented to time, place, and person and is moderately built.

General Physical Examination:

No Pallor

No Icterus

No Cyanosis

No Clubbing

No Lymphadenopathy

No Edema

Vitals:

Temperature: 98.9 F

Blood Pressure: 124/78 mm Hg

Pulse: 70 bpm

Respiratory Rate: 17 cycles per min

Head to toe examination

Spine appears normal

Thyroid appears normal

Breast examination shows normal changes of pregnancy.

Abdominal examination:

Inspection

Shape of the abdomen is globular, it is longitudinally distended

Corresponding quadrants move equally with respiration

Umbilicus is central and everted, flanks are not filled

Linea nigra present

No scars, sinuses or dilated veins can be seen

Hernial orifices are intact

Palpation

No local rise of temperature or tenderness

Abdominal girth is 70cm, at the level of umbilicus

Auscultation

Fetal heart sounds appreciated on dopplers.

Systemic Examination:

CNS:No focal neurological deficits

CVS:S1 and S2 heard and no murmurs heard

RS:Normal vesicular breath sounds heard.

P/A:No hepatosplenomegaly and non tender abdomen

Investigation: 24 hour OGTT

Fasting Plasma Glucose=106gm/dl
2 hr value=138gm/dl

HbA1c-6.3%

24 year old primigravida with 26 weeks 6 days of gestation came for regular antenatal visits who is not a known case of diabetes mellitus prior to conception, found to have increased blood glucose level upon evaluation suggestive of gestational diabetes mellitus .She also has a family history of type 2 diabetes mellitus.

Diagnosis: Gestational Diabetes Mellitus

Dietary Advice given

Thank You